Graphic Era Institute of Medical Sciences

Dhoolkot , Chakrata Road , Dehradun

Billing manual

2024-2025

GEIMS BILLING MANUAL

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Note:-**There are some hospital administrative charges which are applicable for allpatients, for exceptions refer serial no.**

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1 11 AM Discharge

1	Billing Cycle is from 11 am to 11 am.
2	Discharge must be acknowledged by 11:00 am by billing staff. GEIMS Hospitals have also adopted a policy of Grace Period of 2Hours.
3	0 – 8 hrs. of stay shall be considered as half day.
4	More than 8 hrs. of stay shall be considered as full day.
5	Exceptions are: -
	CGHS Patients and CGHS tariff-based PSU patients
	Day Care Packages
	Medical and Surgical Management for less than 24 hrs.

2 Advance Deposit policy

Cash Patient	
1	For Planned procedures 100% of the estimate amount to be collected as advanced.
2	If 100% of the estimated amount has not been cleared / deposited, then surgery clearance shall not be given. Surgery clearance shall be issued post the deposit of requisite advance.
3	In all surgical cases where inappropriate advance is deposited by patient approval is must from GM-Ops for surgery clearance. And in case of Undertaking given by the Consultant that time also need to take approval from GM-Ops.
4	In package Cases, entire package cost is to be taken as Advance before OT clearance.
5	Outstanding amount for patients is not allowed if it is more than 20% of the total bill amount. Keep 20% difference always between outstanding and total bill amount.
TPA Patient	
1	In cases where TPA full approval has been received- minimum Rs 5000/- for Corporate Policy holder and 10000/- for Individual policy holder is to be taken against the non-payable items and In cases where approval is not received then it shall be treated as a Cash patient till approval from TPA and 100% of the estimated expenses to be taken as an advance and for Cases, 100% entire package cost is to be taken as an Advance.
2	In cases where approval received is lesser than the estimate provided- differential amount to be collected at the time of Admission (80% of the total estimate or the Minimum advance against Room category) and for surgical cases differential amount to be collected at the time of Admission.
3	In cases where approval is not received then it shall be treated as Cash patient till approval from TPA and 100% of the estimated expenses to be taken as an advance.
4	In case of denial from TPA during hospitalization, then prevailing hospital rates shall be levied.
5	If patient is not ready to wait until final approval from TPA, patient shall be discharged/settled as per the cash tariff with undertaking note.

3	Ambulance Billing policy

1	Ambulance charges shall be billed to all patients those who are using GEIMS Ambulance and charges as per the tariff list.

4	Assistant Surgeon Fees

1	In case surgery with category High Risk & Assistant surgeon different/Same specialty is assisting, an additional charge shall be levied for the Assistant Surgeon's Fee, wherever it's applicable.
2	Following percentage shall be applicable for Assistant Surgeon's Fee: -
	30% of Primary Surgeon's fee for same specialty
	50% of Primary Surgeon's fee for different specialty

5	Bipap Charges

1	One time Initiation of Bipap shall be billed in all cases that require Bipap.
2	Charges shall include intubation, initiation and Bipap.
3	Tubing and other consumables shall be charged as per actuals.
4	Following billable items shall be applicable: -
	Bipap - Per Day, Initiation of NIV/BIPAP (including BIPAP)
5	Initiation charges are not applicable for CGHS based tariff patients.

6	CGHS Billing (Do's and Don'ts)

1	Only in the case of CGHS/PSU patients, or as advised by Sales Head, the Implant's Vendor Invoiceshall be provided for billing. In all other cases no such invoice is to be shared with patient or payor.
2	Highest priced package shall be billed @100% and rest all shall be charged @50%.
3	For CGHS Oncology where we must follow open billing model of Tata Memorial, a Octarate service has been created in HIS where rates @100% and @50% have been updated
4	For CGHS billing (other than CGHS Oncology), CGHS code and price must be mapped against GEIMS Code in HIS. In case price or code are not appearing correctly, please inform IT Parameters team immediately.
5	Billing cycle should be 24hrs calculation.
6	Vendor Invoice required for all implants and consumable those are outside the package.
7	Equipment's are not payable like ALPHA Bed, DVT pump, Syringe Pump etc.
8	If patient is in ICU, Initiation charges and ECG are not payable.
9	In ICU Oxygen charges are payable as per hourly bases.

10	Where there is no CGHS price for any procedures/investigation, rate would be applicable according to AIIMS rate.
11	If PTCA failed, then CAG package shall be billed to patient and all PTCA related services shall be bill as a part of package CAG.

Others	
1	In case of Road Traffic Accidents and odd hour emergencies the same should not be implemented for first 24 hours, if the customer has a valid TPA ID card as our agreement mandates that.
2	Corporate Patients / corporate being a TPA: No advance for the procedure/surgery should be taken for the treatment. A consent letter should be signed by the concerned corporate employee, where a consent to be taken by stating 'In case TPA denies the payment, entire payment will be cleared by the patient.
3	Cases where denial comes from TPA after the initial authorization should be tracked and escalated to Sales and Marketing so that those can be taken up appropriately with respective TPAs.
4	Proper audit of all the documents should be done before the same should be faxed to TPA's because majority of the denials come due to discrepancy in pre-auth and discharge summary or change in the line of treatment which is not explained during the treatment of the patient.

For Emergencies and non-procedure observation patients following advances to be taken, (Unless the consultant fills an estimate in this case 80% of estimate):

Suite	Rs. 25,000/-
Classic Deluxe	Rs. 20,000/-
Single	Rs. 20,000/-
Double	Rs. 15,000/-
Economy	Rs. 10,000/-
ICU	Rs. 25,000/-
NICU/PICU	Rs. 25,000/-

7 Checklist for Billing Audit

1	Check list for IPD Billing Audit to be used at the time finalizing the bill.
2	Check list is ensuring the accuracy of final bill while preparing.

8	C-pap Charges

1	One time Initiation of C-pap shall be billed in all cases that require C-pap.
2	Charges shall include intubation, initiation and C-pap.
3	Tubing and other consumables shall be charged as per actuals.
4	Following items shall be applicable: -
	C-pap - Per Day, Initiation of CPAP
5	Initiation charges are not applicable for CGHS based tariff patients.

9	Dietician Consultation

1	Diet & Dietician Charges shall be billed once in every day of hospital stay to all WARD IPD patients (S/L, D/L, E/C) But not applicable to All ICU's Patient Except HDU.
2	Exceptions are: -
	Day Care Patients, All ICU's where patient is in Ventilator and Bipap.
	CGHS Patients and CGHS tariff-based PSU patients, All Package cases.

10	Discount applicable for GEIMS staff in Triage	
	Services	Discount 10% FLAT
1	Triage Bed charges	10%
2	EMO Charges	10%
3	Consultation	10%
4	Investigation	10%
5	Procedures	10%
6	Drugs and Consumables	10%

11	Equipment Policy

1	Any in house (GEIMS) equipment will be billed only once to the patient according to higher surgeon fee. In case, multiple in-house equipment's have been used for different specialty both will be billed to the patient.
2	In case of multiple surgeries, the special equipment charges will be charged as per the higher Surgeon fees.
3	Equipment's in post operation monitoring up to 2 hrs. will not be charged.
4	If two equipment's (GEIMS equipment and outside GEIMS equipment) are used in same surgery, then patient will be billed for both (for GEIMS equipment as per pricing policy and outside equipment on actual orunless specified otherwise).
5	If the surgeon is using two equipment (personal equipment & GEIMS equipment) then the patient shall be billed for one equipment as per the pricing policy and charges of the equipment shall be shared equally between the Hospital and Surgeon.
6	In case of multiple surgeries, the special equipment charges will be charged as per the higher Surgeon fees only.

7	If C-Arm used in neuro surgeries, the C-Arm charges shall be billed on actual basis i.e. C - Arm less than 1 Hour and C - Arm more than 1 Hour.
8	If laser/Robotic is used for any surgical procedure it should be billed in addition to OT equipment charges.

12 Fetal Monitoring Charges

1	Fetal monitoring charges up to 9 hours before the delivery shall be Complementary but anything more than 9 hours shall be billed an actual, on per hourly basis.
2	Following items shall be applicable: -
	Fetal Monitoring per hour

3	Exceptions are: -
	GIPSA/TPA - Charges to be collected from patient as non-payable item with an undertaking.
	PSU/Govt/CGHS - not to be billed

13 High Risk and Complex Charges

1	High Risk and complex charges to be charged 20% additionally of surgery cost (in open billing) and 20% extra of package.
2	If any high risk and complex surgery is performed in emergency hours, general holidays or Sundays, only high risk and complex charges will be applicable.

14 ICU rent difference in Package

1	If Patient stay would be required in ICU more than defined LOS in package days, then ICU difference charges shall be applicable.
2	Exceptions are: -
	PSU CGHS based tariff, Special TPA Packages

15 ICU/Ward Nursing Charges

1	Ward Nursing Charges shall be billed to all patients staying in hospital except for patients in Neonate-ICU & Peads-ICU, HDU, NICU,CCU,NSICU.
2	Charges shall be applicable for every IP admission including TPA, PSU on GEIMS Tariff, Corporates, GIPSA and Govt cases routed through TPAs. And if patient get discharge before 01:00PM said charges shall not be applicable on that day.
3	These charges are applicable once in a day and shall be collected from patient as non-payable item.
4	Exceptions are: -

	Newborn baby in GEIMS Hospital,
	CGHS patients and Package days and TPA Daycare cases (AND AS PER AGREEMENT)

16 Medical History Assessment

1	Medical History Assessment is basically MRD charges and shall be billed to all IPD patients for every admission i.e. Generation of a new IPID.
2	Same is also applicable for all regular IP admission except Daya care.
3	Only one Medical History Assessment shall be billed for procedures that require Multiple sittings or admissions like Radiotherapy or IVF.
4	MRD charges shall not be applicable for patients re-admitted within 24hrs of a discharge
5	Medical History Assessment to be waived off for Baby less than a month old, even if the born was outside GEIMS Hospital.
6	Exceptions are: -
	Triage (for Non admissions or direct discharges from Triage only), Pre-ADT (till admission is not done) Day Care
	Newborn Baby in GEIMS Hospital, GEIMS Employees & Dependent, CGHS Patients and CGHS tariff-based PSU patients.

17 MLC Charges

1	In MLC cases an additional charge shall be levied to cover extra cost incurred by hospitals. This shall only be applicable for MLC cases where a patient gets admitted to our hospital and applicable for Outside Registered MLC Cases. This shall not be applicable for cases where there is direct discharge from Triage/ER.
2	In Death cases, MLC charges shall not be billed to the patient.
3	Exceptions are: -
	CGHS/PSU Patients

18 Mortuary Related Charges Policy

1	Mortuary charges, Body Wrapping Sheet and Patient Gown shall not be billed to any patient irrespective of any channel.

19 Nebulization Charges

1	Nebulization charges shall be billed as per actual usage.
2	In case of any ICU/HDU stay, Nebulization charges shall be billed up to 5 times in 24 hrs.
3	Exceptions are:-
	During OT and Post-Operative observation

20	Needle Stick Injury Discount Policy

1	Needle Stick Injury discount scheme is applicable only for hospital staff wherein they are entitled for 100% discount on some specified investigations.

21	OPD Procedures Policy

1	If a procedure is done in OPD then both the Consultation and Procedure charges can be charged.
2	Any consumption used during an OPD Procedure should be billed to patient as per actuals.

22	OPD/IPD Consultation Policy

1	In case a patient is being treated by more than one specialist, the Primary treating doctor shall charge IP consults twice a day and Secondary treating doctor shall charge consults once a day. But One (each) emergency consult in a day is allowed for both Primary and Secondary doctor as per GEIMS policy.
2	One follow-up OPD Consultation with 100% discount within one week of a surgical /Medical case shall be billed as"First free follow-up consult after discharge" and Post Discharge Suture & Staple removal will be complimentary but consumables will be charged on actual.
3	All patients coming for follow up in OPD within 3 days (to discuss the reports) shall be billed as zero consult, consultation billing should be considered as 100% discount.
4	Due to nature of treatment, there shall be no free follow-up consultation in OPD in departments of Psychiatry and Psychology.
5	For all admissions after 12 Pm only One IPD Consult shall be charged to patient.
6	**IP Consultation Policy for CGHS patients: -**
	a) Admitting Doctor - Per day 2 consults of admitting doctor's specialty.
	b) Cross Referral Doctor - as per CGHS guidelines only 2 visits are allowed in a day whether it is admitting doctor visit or cross referral.
7	**IP Consultation Policy for CASH/TPA/Corporate/PSU (**GEIMS **Tariff)/international patients: -**
	a) Admitting Doctor - Per day 2 consults of admitting doctor's specialty.
	b) Admitting Doctor (Emergency Hour Consult) - Only 1 per day of admitting doctor's specialty during emergency hours and 1 emergency visit shall be applicable for cross referral specialty up to 5.
	c) Cross Referral Doctor - Only 1 consult per day per specialty up to 5.
	d) Intensivist - Per day 2 consults of Intensivist in ICU and 1 for HDU only.
	c) More than one Admitting Doctor - Per day 2 consults of each admitting doctor's specialty.
	e) Post-operative Visit - After surgery within 24 hrs. no consult for surgeon.
8	Additional consult to be charged for Radiologist or Cardiologist who performs a Radiology or Cardiology test during Emergency hours, general holidays or Sundays.

9	Daycare surgical cases there is no consult for surgeon, if any cross referral done for patient shall be levied.

23 Opthal Lens / Implant Difference

1	In general, if patient opts for a superior quality of Lens/Implant/Stent whose cost is more than the Ceiling limit prescribed by TPA/GIPSA/PSU/Company, then patient must pay the difference of selling price and ceiling rate of implant.
2	But if implant is approved for PSU/Govt, then additional charges equal to difference of vendor invoice rate and ceiling rate of implant has to be borne by patient.
3	If implant is not approved for PSU/Govt, then additional charge equal to difference of selling price and ceiling rate of implant has to be borne by patient.

24 Oxygen Charges

1	In ICU, Oxygen charges shall be considered as a part of ICU room rent but if it is being used in Ward/Non ICU Bed, it shall be billed Octarately.
2	Following items shall be applicable: -
	Oxygen charges per hour, Oxygen charges between 4 - 12 hours and Oxygen charges (24 Hour)

25 Package Billing

1	If package is available for particular surgery or procedure in HIS or Tariff, then it shall be billed to patient irrespective of channel of patient.
2	There is no such policy to bill (only) Open Surgery to TPA and Package to Cash patient for same surgery.
3	Consumption limits defined in tariffs or HIS for packages must be strictly followed.
4	For any exception billing in package please take prior approval of Hospital Operations Head.
5	In case of multiple packages in same sitting and different body site, both packages shall be billed @100%.
6	In cases of multiple packages in same sitting and same body site, the highest priced package shall be billed@100%, second package 75% and third package onwards 60%.
7	In case of multiple packages in different sitting (irrespective of body site) both packages shall be billed at 100%.
8	In case patient opted for higher room category after completing package LOS, package price shall not be changed to higher room tariff of opted room category, rest all charges are applicable opted room category after package LOS.
9	In case of Complex Angioplasty and Coronary Angiography done in same sitting, then CAG shall be charged 75%.
10	In case of a failed PTCA, if patient is immediately transferred for CABG, then only consumables related to PTCA shall be charged to patient and not PTCA package.

26	Patient Category

1	Category A: EWS, Free Patients
2	Category B: CGHS, ECHS, BGSSS, PGEPHIS, Govt. Schemes (TPA Routed), Govt. or Any Package Patient irrespective of his channel
3	Category C: Cash, TPA, Corporate, International
4	Patient category is based on the tariff and not on the mode of payment (cash or credit) of settlement.
5	In case a patient was admitted on hospital tariff but later is unwilling or unable to pay for its bills, the category of patient should be changed to Category B.

27	Patients convert from TPA to Cash

1	If any TPA patient convert from TPA to Cash (due to any reason), all charges shall be billed as per current cash rate.

28	Patient Diet Charges

1	Diet Charges shall be billed once in every day of hospital stay to all IPD patients for every admission (Not applicable if patient get discharge before 01:00 PM and stay is > 24 Hours).
2	Exceptions are GEIMS Day Care patients where diet services not availed, CGHS/PSU Patients, Newborn Baby in GEIMS Hospital, Children below the age of 2 years, Package Stay patients, All ICU's where patient is on Ventilator, Bi-pap and Liquid diet.

29	Random Blood Sugar by Glucometer

1	Random Blood Sugar test by Glucometer shall only be charged up to 12 times in a day.
2	Any number of tests more than 12 shall be considered non chargeable.

30	Re-Exploration Surgery policy

1	For REDO Surgery within 48 hrs., only consumables shall be charged (no extra cost for Surgeon Fee, OT or Anesthesia) with prior written approval from Director/ Head of Department/Departmental Coordinator.
2	For Redo surgeries after 48 hrs-72 hrs., 50% of surgeon fee and percentage shall be applicable on all subsequent areas i.e. (OT Charges, Anesthesia Charges). However, consumables & pharmacy will be charged as per actuals.
3	For Redo surgeries after 72 hrs., 100% of surgeon fee. However, OT charges, Anesthesia fee, consumables & pharmacy will be charged as per actuals. This can be waiver strictly with approval of GM Ops.

31	Refund Policy

1	Refund of a bill shall not be done by the same GEIMS User who has issued the original bill or receipt.
2	All refund bills should be counter signed by Supervisor, Duty Manager or FO Head.
3	While issuing the refund it is mandatory to collect the original bill receipt from patient along with their signature, reason for refund and Patient contact number.
4	Following is the refund process of International Patients: -
	1. Refund money shall only be given to patient.
	2. No refund shall be given to any GEIMS Employee.
	3. Refund would require validation and signature of Unit Finance Head.
	4. Needless to mention that every patient who has availed any service in our hospital must be given a copy of Invoice or Bill.
32	Registration Charges

1	A one-time registration charge Wil be complementary for all patients coming to GEIMS hospital for the first-time.
2	Whenever a new UHID is being generated for a patient, the registration charge shall not be applicable, even if patient directly comes of IPD.

33	RMO Fees/Intensive Visit

1	RMO Fees shall be billed only once in a day to all IPD ward patients.
2	RMO Fess are also same as other consultation fees for PSU companies which are already fixed a price for consultation.
3	Intensive Visit Fees shall be billed only once in a day to all IPD ICU patients.
4	Exceptions are: -
	PSU/CGHS Patients
	HDU/NICU/PICU stay
	Package Days and Newborn baby in GEIMS Hospital

34	Surgeries performed in Emergency hours, General Holidays or Sundays

1	Surgeries performed in emergency hours, general holidays or Sundays to be charged at 20% (Of total surgery cost) extra for open billing and for packages 15% (Package cost) extra (other than planned)
2	Exceptions are:-

	Normal Delivery and Caesarean Section, CAG and PTCA are exempted from emergency charges

35 TPA Billing Policy

1	Non-payables items should be taken from patient before discharge and explain it to them.
2	Non-payables items list needs to send along with final bill to TPA's for final approval.
2	Collect non-payables from patient as per IRDA guidelines until and unless TPA's approval says not to collect these from patient.
3	Calculate discount to TPAs as per MOU and after deducting cost of Non payables, Co-payment, room or surgery difference and other patient share. Mark all non- payables items through HIS except package cases.
4	Discounts should be given to agreed TPA/Insurance companies before settlement of bill in HIS.
5	In case of denial from TPA/Insurance during hospitalization, then billing shall be done as per prevailing cash tariff of hospital.
6	If a package is available for a surgery, then you must bill package even if doctor advice for open surgery billing.
7	Ensure right TPA and Company name are selected in HIS.
8	Initial approval shall not be considered until final approval is received from TPA and all final bills should be informed to concerned patient TPA's.
9	If a patient does not wish to wait for final approval from TPA/Insurance, then the patient's bill shall be prepared and settled as per prevailing cash tariff of hospital. Also take an undertaking from patient which is attached here-in for reference. This rule is also applicable even if TPA's initial approval say that this is full/final approval as without discharge summary and final bill the case is not considered closed by TPAs.
10	In case patient chooses better implants than stipulated by TPA/Insurance, please collect differential from patient along with an undertaking. Undertaking format Annexure 1.

36 Triage Billing Policy

1	Up to 8 hrs. to be charged per hourly basis
2	Beyond 8 hrs. as per opted bed category by patient
3	All cash patients who are getting discharge directly from triage and their stay is less than 8 hours, shall be charged as per OPD rates for Investigations and Procedures.
4	In case patient has not opted for bed category with in 8hrs, then all existing charges shall be converted as per economy room type.

37 Ventilator Charges

1	One time Ventilator of Initiation charges shall be billed in all cases that require ventilator.

2	Charges include intubation, initiation and ventilation.
3	No extra charges for ventilator if Patient is on Isolation bed.

TPA undertaking Format for cash settlement

To,
GEIMS Hospital,
Unit Location

Subject: Undertaking to Settlement of Bill in Cash rate

Sir/Madam,

1. I_____S/o_____,IP No._____Aged about _____ R/o_____is a '_____Beneficiary' under Employee Medical Health Scheme, to whom valid Medical Facility Card bearing no._____been issued TPA.

2. I am being treated in GEIMS HOSPITAL Dehradun for___.

3. I am aware with the terms and conditions of my Medical Policy as provided by my Company.

4. I understand the whole process of TPA discharge, and I am not ready to wait till approval from TPA, so I want to settle my bill as per prevailing cash tariff of hospital.

5. I am in the proper state of mind to give above undertaking. I have read and understood aboveundertaking, and it has my free consent without any pressure whatsoever.

Signature:_____ Date: _____
Name:_____ Place: _____
S/o _____
Address: _____

Witness 1 :
Name :
Address :

Witness 2 :
Name :
Address :

Undertaking Format for Len/Implant/Device

To,
GEIMS Hospital,
Location

Subject: Undertaking to bear the difference cost of any implant used, which has a rates ceiling fixed byPSU/ TPA.

Sir/Madam,

1. I_____S/o_____, aged about_____R/o_____is a'_ _____Beneficiary' under Employee Medical Health Scheme, to whom valid Medical Facility Card bearing no._____been issued by PSU/TPA.

2. I am being treated in GEIMS HOSPITAL_____for_____.

3. I am aware with the terms and conditions of my Medical Policy as provided by my Company.

4. I understand that there are rate ceilings fixed by PSU/TPA on the cost of implants to be used intreatment. All the over and above cost has to be borne by the patient himself.

5. I agree and undertake to bear the difference cost of any implant used while treatment by Hospital,which has rate ceiling fixed by PSU/TPA and will not take any re-imbursement/claim from PSU/Insurance Company/Hospital. I undertake to make the above payments to hospital as and when demanded byHospital

6. I have understood and agreed that for_____implant I have to pay_____amount as a difference cost to hospital in advance directly.

7. I authorize the Hospital to use the said implants for my treatment. I agree and understand to bear suchdifference cost without claiming from PSU/Insurance Company/Hospital.

8. I am in the proper state of mind to give above undertaking. I have read and understood above undertaking,and it has my free consent without any pressure whatsoever.

Signature:_____ Date : _____

Name:_____ Place: _____
S/o _____
Address: _____

Witness 1 : Witness 2 :
Name with add: Name :